


Laboratory Investigation Report

Patient Name	: Mr. Vipin Kumar Garg	Centre	: 4736 - Mr. Ravinder Pal Singh
Age/Gender	: 68 Y 7 M 0 D / M	OP/IP No/UHID	: //
MaxID/Lab ID	: ML03424033/4018062600052	Collection Date/Time	: 29/Jun/2026 03:30PM
Ref Doctor	: SELF	Reporting Date/Time	: 01/Jul/2026 04:28PM

SEROLOGY SPECIAL.


SIN No: B2B9344524

Test Name	Result	Unit	Bio Ref Interval
Interferon gamma release assay (IGRA), Special Tube			
Nil Control	1.29	IU/mL	
TB Antigen 1	8.00	IU/mL	
TB Antigen 2	9.60	IU/mL	
Mitogen	4.97	IU/mL	
Final Result	Positive		

Interpretation :

Nil (IU/ml)	TB1 minus Nil IU/ml)	TB2 minus Nil IU/ml)	Mitogen minus Nil (IU/ml)	QFT plus result	Interpretation
<=8	>=0.35 and >25% of nil value	Any	Any	Positive	<i>M tuberculosis</i> infection likely
<=8	Any	>=0.35 and >25% of nil value	Any	Positive	<i>M tuberculosis</i> infection likely
<=8	<0.35 or >=0.35 and <25% of Nil value	<0.35 or >=0.35 and <25% of Nil value	>=0.5	Negative	<i>M tuberculosis</i> infection NOT likely
<=8	<0.35 or >=0.35 and <25% of Nil value	<0.35 or >=0.35 and <25% of Nil value	<0.5	Indeterminate	Likelihood of <i>M tuberculosis</i> infection cannot be determined
>8	Any	Any	Any	Indeterminate	Likelihood of <i>M tuberculosis</i> infection cannot be determined

Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017

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Booking Centre : 4736 - Mr. Ravinder Pal Singh, Booth No. 227, Sector 21, 9646425103

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MC-2714

**Laboratory Investigation Report**

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SEROLOGY SPECIAL.

SIN No: B2B9344524

Test Name	Result	Unit	Bio Ref Interval
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Comments:

1. This assay intended for use in conjunction with clinical findings and other diagnostic tests for tuberculosis and should not be used as a sole diagnostic assay for diagnosis of tuberculosis.
2. False-positive results may be seen in patients with prior infection with other Mycobacterium species like M marinum, M szulgai, or M kansasii.
3. The magnitude of the measured IFN- γ level cannot be correlated to stage or degree of infection, level of immune responsiveness, or likelihood for progression to active disease
4. False-negative results can be due to stage of infection (e.g., sample taken prior to the development of cellular immune response), co-morbid conditions that affect immune functions, low lymphocyte counts, reduced or absent activation of immune response to TB antigens or other immunological variables. CDC recommends repeat test after 8 - 10 weeks in case of high suspicion of tuberculosis.

Kindly correlate with clinical findings

***** End Of Report *****

Dr. Poonam S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Bansidhar Tarai, M.D
Director, Microbiology & Molecular
Diagnostics



Dr. Rup Jyoti Chandak, M.D.
Sr. Consultant, Microbiology



Dr Nidhi Malik, MD
Consultant Microbiology

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MC-2714



Postgraduate Institute of Medical Education and Research, Chandigarh

Department of Haematology (EMERGENCY LABORATORY)

HAEMOGRAM REPORT

Patient Name V K Gurg Lab ID No. H702
Age/Sex 68 Yr/M Requisition date 27/06/2026
CR number 201405136462 Diagnosis
Dept./Unit Private Ward 4th D

	Result	Reference Range
Haemoglobin, Hb (gm/dL)	11.3	13.5 - 17.5
Haematocrit / PCV (%)	34.0	41 - 53
Red cell count, RBC ($\times 10^{12}$ /L)	3.52	4.5 - 5.9
Mean Corpuscular Volume, MCV (fl)	96.6	80 - 100
Mean Corpuscular Haemoglobin, MCH (pg)	32.1	26 - 34
Mean Corpuscular Hb Conc., MCHC (gm/dL)	33.2	31 - 37
Red Cell Distribution Width, RDW-CV (%)	15.6	11.0 - 14.5
Reticulocyte Count (%)	-	1.0 - 2.7
ESR (mm in 1 st hour)	-	0 - 13
Platelet Count ($\times 10^9$ /L)	152	150 - 450
Total Leucocyte Count, TLC ($\times 10^9$ /L)	7.50	4.0 - 11.0

Differential Leucocyte Count					
	Result	Reference Range		Result	Reference Range
Neutrophils (%)	-	41 - 72	Absolute Neutrophil Count ($\times 10^9$ /L)	-	1.8 - 7.7
Lymphocytes (%)	-	15 - 45	Absolute Lymphocyte Count ($\times 10^9$ /L)	-	1 - 4.8
Eosinophils (%)	-	0 - 6	Absolute Eosinophil Count ($\times 10^9$ /L)	-	0.1 - 1
Monocytes (%)	-	2 - 10	Absolute Monocyte Count ($\times 10^9$ /L)	-	0.2 - 1
Basophils (%)	-	0 - 1	Absolute Basophil Count ($\times 10^9$ /L)	-	0 - 0.2
Blasts (%)	-				
Promyelocytes (%)	-				
Myelocytes (%)	-				
Metamyelocytes (%)	-				
nRBC (per 100 WBC)	-				

Peripheral Smear / Comments

Note: This report is being released without smear review. Results should be interpreted only by authorized personnel. Please contact the lab, if required.

Advice

-

Validated On

27-06-26 02:27 PM

Validated By

-(-)



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Biochemistry

Laboratory :Indoor Clinical Biochemistry Lab

Sample :Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 68 Yr/M

Dept-Unit : Clinical Hematology & Medical Oncology-Hematology
Clinic

Req. Date : 27/06/2026

Ward : Private Ward 4th D

Room No: Room 36

Bed: Bed 36

Clinician :

Lab Sample No. : 15474

Diagnosis :

Investigation	Result	Unit	Standard Ranges
Sodium	137.2	mmol/L	135 - 145
Potassium	4.50	mmol/L	3.5 - 5
Chloride	107.9	mmol/L	90 - 107
Urea	35.8	mg/dL	10 - 50
Creatinine	1.63	mg/dL	0.5 - 1.2
Phosphorus	2.37	mg/dL	2.7 - 4.5
Calcium	7.96	mg/dl	8.8 - 10.2
Protein total	6.41	g/dL	6.4 - 8.3
Albumin	4.36	g/dL	3.4 - 4.8
Bilirubin (total)	0.20	mg/dL	0.2 - 1.2
Bilirubin (conjugated)	0.09	mg/dL	0 - 0.3
Uric Acid	7.8	mg/dl	3.4 - 7.4
AST	18.7	U/L	2 - 40
ALT	13.9	U/L	2 - 41
ALP	95	U/L	42 - 128
Mg	2.18	mg/dL	1.58 - 2.55
CRP	1.78	mg/l	0 - 5
LDH	152	U/L	135 - 225

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MaxID/Lab ID	: ML03424033/4018062600043	Collection Date/Time	: 27/Jun/2026 03:05PM
Ref Doctor	: Dr.Pankaj Malhotra Pgi	Reporting Date/Time	: 28/Jun/2026 10:41AM

Clinical Biochemistry

SIN No: B2B8387735

Test Name	Result	Unit	Bio Ref Interval
IgG (Immunoglobulin IgG), Serum			
Immunoglobulin IgG	756.0	mg/ dL	700-1600
Immunoturbidimetric			

Interpretation

Polyclonal increase seen in autoimmune diseases (systemic lupus erythematosus, rheumatoid arthritis, Sjögren's syndrome), sarcoidosis, chronic liver disease, some parasitic diseases and chronic or recurrent infections.

Monoclonal gammopathies seen in IgG type multiple myeloma, lymphomas, leukemia, and other malignancies.

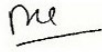
Hypogammaglobulinemias, IgG deficiency may be genetic as in severe combined immunodeficiency or acquired as in AIDS. A decrease in IgG also occurs as a result of thermal burns, nephrotic syndrome, protein losing enteropathies and non-IgG myelomas.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Director & Quality Manager



Dr. Rajeev Kumar, DCP, MD
Associate Consultant
Biochemistry

Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017

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Ref Doctor	: Dr.Pankaj Malhotra Pgi	Reporting Date/Time	: 27/Jun/2026 05:47PM

Clinical Biochemistry



SIN No:B2B8387735

HbA1c (Glycated/ Glycosylated Hemoglobin) Test, EDTA

HPLC

Date	27/Jun/2026 03:05PM	20/Aug/24 10:39AM	25/Jun/24 10:18AM	11/Jun/24 09:52AM	28/May/24 11:34AM	Unit	Bio Ref Interval
Glycosylated Haemoglobin(Hb A1c) HPLC	5.70	4.30	4.80	5.20	5.30	%	4.27 - 6.07
Glycosylated Haemoglobin(Hb A1c) IFCC Calculated	38.78	23.48	28.95	33.32	34.41	mmol/mol	< 39.0
Average Glucose Value For the Last 3 Months	116.89	76.71	91.06	102.54	105.41	mg/dL	
Average Glucose Value For the Last 3 Months IFCC Calculated	6.47	4.25	5.04	5.68	5.84	mmol/L	

Interpretation The following HbA1c ranges recommended by the American Diabetes Association(ADA) may be used as an aid in the diagnosis of diabetes mellitus.

HbA1C(NGSP %)	HbA1C(IFCC mmol/mol)	Suggested Diagnosis
≥ 6.5	≥ 48	Diabetic
5.7 - 6.4	39 - 47	Pre- Diabetic
< 5.7	< 39	Non - Diabetic

HbA1C provides a useful index of average glycaemia over the preceding 6-8 weeks.

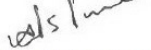
It is suggested that HbA1c is measured every 6 months in stable patients, every 3 months in patients with unstable metabolic control and every month in pregnancy. Increased Glycated hemoglobin is a reflection of Hyperglycemia.

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Anjali Bhutani, MD
Sr Director & Head, Max Lab


Dr. Neerja Dhangra, MD, DNB
Associate Director, Max Lab


Dr. Ajit Pratap Singh Panaych, MD
Consultant, Pathology

Test Performed at :1066 - Max Hospital Mohali, Near Civil Hospital, Phase-6,Mohali

Booking Centre :4736 - Mr. Ravinder Pal Singh, Booth No. 227, Sector 21, 9646425103

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MC-2287



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MaxID/Lab ID	: ML03424033/4018062600043	Collection Date/Time	: 27/Jun/2026 03:05PM
Ref Doctor	: Dr.Pankaj Malhotra Pgi	Reporting Date/Time	: 28/Jun/2026 01:14PM

Serology



SIN No: B2B8387735

Test Name	Result	Unit	Bio Ref Interval
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Hep B Core Antibody-Total, Serum

CLIA

Hep B Core Value	3.13	S/CO
CLIA		

Ref. Range

Positive <0.99 It indicates a reactive sample and the presence of anti-HBc.
 Borderline 1.00 - 1.19 It indicates a borderline sample.
 Negative >=1.2 It indicates a non reactive sample, negative for anti-HBc.

Interpretation

This assay is useful for diagnosis of recent or past hepatitis B Infection.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Bansidhar Tarai, M.D.
Director, Microbiology & Molecular
Diagnostics



Dr. Rup Jyoti Chandak, M.D.
Sr. Consultant, Microbiology



Dr. Nidhi Malik, MD
Consultant Microbiology

Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017

Booking Centre : 4736 - Mr. Ravinder Pal Singh, Booth No. 227, Sector 21, 9646425103

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MC-2714



Laboratory Investigation Report

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Ref Doctor	: Dr.Pankaj Malhotra Pgi	Reporting Date/Time	: 27/Jun/2026 05:26PM

Serology



SIN No: B2B8387735

Test Name	Result	Unit	Bio Ref Interval
HCV IgG Antibody (Hepatitis C Virus), Serum			
CLIA			
HCV, IgG	Non Reactive		
CLIA			
HCV, IgG Test Value	0.03	S/CO	< 0.90

Ref. Range

Negative < 0.90
Borderline 0.90 - 8.0
Positive > 8.0

Interpretation

This test is a screening test performed on VITROS immunodiagnostic system using immunometric technique.

- Hepatitis C (HCV) is an RNA virus of Flavivirus group transmitted via blood transfusions, transplantation, injection drug users, accidental needle punctures in healthcare workers, dialysis patients and rarely from mother to infant.
- 10% of new cases show sexual transmission. As compared to HAV & HBV, chronic infection with HCV occurs in 85% of infected individuals.
- This test is indicator of past or present infection, but does not differentiate between Acute / Chronic / Resolved infection. HCV RNA PCR recommended in all reactive results to differentiate between past and present infection
- A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Manpreet Kaur, MD
Sr Consultant, Microbiology

Test Performed at : 1066 - Max Hospital Mohali, Near Civil Hospital, Phase-6, Mohali
Booking Centre : 4736 - Mr. Ravinder Pal Singh, Booth No. 227, Sector 21, 9646425103
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MC-2287

**Laboratory Investigation Report**

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MaxID/Lab ID	: ML03424033/4018062600043	Collection Date/Time	: 27/Jun/2026 03:05PM
Ref Doctor	: Dr.Pankaj Malhotra Pgi	Reporting Date/Time	: 28/Jun/2026 12:48PM

Serology

SIN No: B2B8387735

Test Name	Result	Unit	Bio Ref Interval
Hep B Core Antibody-IgM,Serum			
CLIA			
Hep B Core-IgM	0.02	S/CO	< 0.80
CLIA			

Ref. Range

Non Reactive < 0.80
Borderline ≥ 0.80 - < 1.20
Reactive ≥ 1.20

Interpretation

Hepatitis B core specific IgM class antibody is detected in acute infections and is a reliable marker for acute disease.

Kindly correlate with clinical findings

*** End Of Report ***



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Principal Director-
Max Lab & Blood Bank Services



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Director, Microbiology & Molecular
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Dr. Nidhi Malik, MD
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Ref Doctor	: Dr.Pankaj Malhotra Pgi	Reporting Date/Time	: 27/Jun/2026 04:59PM

Serology

SIN No: B2B8387735

Test Name	Result	Unit	Bio Ref Interval
Hepatitis B Surface Antigen, Serum			
CLIA			
HBsAg	Non Reactive		
CLIA			
HBsAg Test Value	0.07	S/CO	< 0.90
CLIA			

Ref. Range

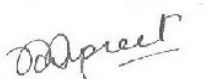
Negative < 0.90
Borderline 0.90 - 5.0
Positive > 5.0

Interpretation

- This test is used to detect hepatitis B surface antigen (HBsAg) in serum sample based on VITROS immunometric immunoassay technique and aid in the laboratory diagnosis of HBV infection.
- Viral hepatitis is a major public health problem with an estimated 257 million persistent carriers of hepatitis B virus (HBV) worldwide. Infection with HBV results in a wide spectrum of acute and chronic liver diseases that may lead to cirrhosis and hepatocellular carcinoma.
- Transmission of HBV occurs by percutaneous exposure to blood products, needle stick injury, sexual contact and perinatally from HBV-infected mothers to baby.
- Hepatitis B surface antigen (HBsAg), derived from the viral envelope, is the first antigen to appear following infection.
- Positive results should be correlated with other potential laboratory abnormalities and clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus.
- Levels of HBsAg may be undetectable both in early infection and late after infection.
- In rare cases HBsAg tests do not detect certain HBV mutant strains.
- HBs Ag disappears with recovery from clinical disease in most patients, however, it persists for years in carriers.

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Manpreet Kaur, MD
Sr Consultant, Microbiology

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MC-2287